



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متكرر

إسم المريض ربيعاً : نورهان حربى عمر على
المحافظة : القليوبية الحي / المدينة : قلوب
رقم البطاقة : 29401190103505 رقم التليفون : 01098109889
العمر (تاريخ الميلاد) : 19-01-1994 المهنة : ربه منزل

الإقرار

أقر أنا الموقع أدناه أنني لا اتمتع بأي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات أو أي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك
الإحصائي الاجتماعي
التوقيع: (-----) المقر بما فيه
التوقيع: (-----)

التشخيص الإكلينيكي : اورام ليمفاوية
نتائج الأبحاث : (مرفق)
التشخيص النهائي : اورام ليمفاوية
خطة العلاج : أورام كيماوي علاج تدعيمي وتلطيفي [6396] قسم العلاج : الاورام
التعليق على التشخيص : -----

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني الوظيفة: ----- التوقيع: -----
الاسم: د/ندى حسن الوظيفة: ----- التوقيع: -----
الاسم: د/نرمين رمضان الوظيفة: ----- التوقيع: -----
أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بأن البيانات المدونة في هذا النموذج صحيحة.
تعتمد صحة البيانات المرفقة
رئيس اللجنة ومدير المستشفى.

تعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

د. / ----- د. / ----- د. / ----- د. / -----

قرار اللجنة

التشخيص النهائي : -----
خطة العلاج : -----
المستشفى : ----- قسم : -----
جهة العلاج : ----- نفقات : -----
ملاحظات : -----

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

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Name: نورهان حري عمر علي National ID: 29401190103505	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته
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تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد أنه
--

A patient of lymphoma for Supportive and palliative care with follow-up

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

عاجل

جمهورية مصر العربية
بطاقة تحقيق الشخصية

نور هان
حري عمر على
٢ ش قيس قلوب المحطة
قلوب - القلوبية

١٩٩٥/٠١ ٢٩٤.١١٩.٠١.٣٥.٥

LA4668591

كيساحي

GEMOX



تدعيم تحاليل وفوضها



٢٠٢٥/٠٢ ٢٩٤.١١٩.٠١.٣٥.٥

ربة منزل

أنثى
معلمة
متزوجة
عبد النبي فهمي حسين محمد

البطاقة مارية حتى ٢٠٢٢/٠٢/٢٤

١١

Patient Name : الأستاذ / نورهان حربي عمر	Request No. : 13002113
Patient No. : 118-2024	Request Date : 08/11/2025
Path No. :	Verification Date : 16/11/2025
Sex : Female	Financial Category : جوسفات روسي - نفقة الدولة
Age : 31 Y	Department : Out Patient
Referring Doc :	Registered by : Eman Hamdi

Bone Marrow Report

BMB (Bone Marrow Biopsy) Report

Clinical data:

Hodgkin lymphoma follow up

BMA comment:

Normocellular for age, normal hematopoiesis, plasma cells 7%, lymphocytes 20%.

Gross examination:

BMB core from R/PSIS, 4 cm, unilateral, multiple cores (2)

Technique: Sections were prepared from paraffin block, processed in Bench Mark ultra machine (Ventara), and stained with rabbit monoclonal antibodies using ultra view HRP DAB as chromogen and hematoxylin as counter stain.

Microscopic examination:

Adequacy: Adequate >7 intertrabecular spaces.

Cellularity: High Normocellular for age (70-80%) cells: fat spaces.

Hematopoiesis: Normal hematopoiesis with normal granulopoiesis, erythroid normally represented with erythroid nests, normal megakaryocytes.

M/E ratio: normal

Fibrosis: MF grade 0/III

Abnormal Infiltration: query granulomas with reactive background composed of histiocytes, plasma cells, eosinophils and mononuclear cells.

IHC:

Diagnosis and Conclusion:

Referred patient as Hodgkin lymphoma follow up, High Normocellular marrow for age, for marker study

Recommendations: Marker study (CD30, CD15) by IHC.

Dr. Henda Abd Elhady, M.D

Ass. Prof. of Hematopathology, NCI
Cairo University.

Verified by :

Name: نورهان حربي عمر علي		GUSTAVE ROUSSY	
MR. : 2538			
Detailed Medical Report			
		السيد	
تجبة طبية وبعد,,			
		إيماء للطلب المقدم من	
بشأن تقرير طبي للمريض			
		بقسم:	
رقم طبي:			
ننشرف بأن نوضح فيما يلي تقريراً عن حالته			
تاريخ وساعة الدخول :		تاريخ وساعة الخروج :	
وبعد الكشف والفحوصات والمتابعة وجد انه			
The patient was initially diagnosed with Hodgkin lymphoma (HD) in 2023 and received first-line therapy consisting of ABVD for four cycles followed by AVD for two cycles. After a one-year remission, disease relapse occurred as stage 4B, prompting second-line treatment with three cycles of ICE. The patient therefore completed four cycles of ABVD, two cycles of AVD, and three cycles of ICE. A PET-CT performed on 24 March 2026 demonstrated metabolic progression of nasopharyngeal mucosal disease (SUVmax 11.5, previously 5) and multiple bilateral submandibular and cervical lymph nodes (largest 1.8 × 1.5 cm, SUVmax 6.1, previously 3.8). Mild progression was noted in mediastinal nodes (largest 2.4 cm, SUVmax 4.3, previously 1.4 cm, SUVmax 3.8). Abdominal lymph nodes remained stable (2.5 × 1.2 cm, SUVmax 6.9, previously 3 × 1.4 cm, SUVmax 5.9). The spleen was enlarged (SUVmax 4.9, previously 5.8). These findings were documented prior to initiation of the first cycle of Gemox. On the same day, the pharmacy recorded the patient's weight (56 kg), height (155 cm), and body surface area (1.5 m²) and prepared Gemzar 1500 mg and Oxaliplatin 200 mg for administration. The patient did not receive the scheduled cycle on 5 April 2026 due to a "qarar" issue, pending laboratory results for cycle 1 day 1 (C1D1) of Gemox. On 5 April 2026 the plan for C1D1 Gemzar (1 g/m²) and Platinum (75 mg) was divided between day 1 and day 2, with hydration, Lasix, and G-CSF support to be given after the cycle. Following the last cycle, the patient reported generalized fatigue; physical examination showed no organomegaly or lymphadenopathy. A dose reduction of 25 % was planned for the next chemotherapy administration, to be postponed for one week. On 21 April 2026 the patient experienced fever at home, with a total leukocyte count of 1.2 × 10⁹/L, lymphocytes 0.2 × 10⁹/L, and hemoglobin 7.3 g/dL, necessitating a blood transfusion. Platelets were 211 × 10⁹/L. The chemotherapy cycle was postponed, a 25 % dose reduction was ordered for the next cycle, and oral antibiotics with G-CSF were prescribed, with follow-up labs to be obtained the following week. The pharmacy note on 28 April 2026 recorded the patient's weight (56 kg), height (155 cm), BSA (1.5 m²) and adjusted body weight (51.1 kg) for C1D15 Gemox with a 25 % dose reduction, substituting Carboplatin AUC 3 for Oxaliplatin due to drug unavailability. On 19 May 2026 the scheduled C22D15 chemotherapy was postponed to the following Sunday after a CBC revealed feverish status with total leukocyte count 1.6 × 10⁹/L and lymphocytes 0.7 × 10⁹/L.			
تحريرا في ,			
		التاريخ : 21-09-2026	
		الطبيب المعالج : hager shahin	
يعتمد ,			

On 24 May 2026 the patient received cycle 2 day 15 of Gemox with a 25 % dose reduction; laboratory results were acceptable except for hemoglobin 6.2 g/dL, for which 500 mL packed RBCs were administered. Premedication included dexamethasone, ondansetron, diphenhydramine IV, pantoprazole, and Septrin prophylaxis.

On 24 June 2026 the patient was given C3D8 Gemzar with prophylactic neutropenic support (Neupogen) for three days.

A PET-CT on 28 July 2026 showed metabolic regression of the nasopharyngeal thickening (SUVmax 4.6, previously 11.5) and regression of supra- and infra-diaphragmatic lymph nodes after completing two additional cycles of Gemzar/Carboplatin. The patient has now completed a total of five cycles of Gemzar/Carboplatin.

On 6 September 2026 the patient, with relapsed Hodgkin lymphoma after five cycles of Gemcitabine/Carboplatin, was noted to be in complete remission except for residual nasopharyngeal disease. Physical examination was unremarkable, and the patient was scheduled for day 15 of cycle 5 (C5D15) of Gemzar 800 mg/m² with referral for autologous stem cell transplantation. The patient has completed five cycles of Gemzar/Carboplatin.

Patient Name : الأستاذ / نورهان حربي عمر	Request No. : 13002112
Patient No. : 118-2024	Request Date : 08/11/2025
Path No. :	Verification Date : 09/11/2025
Sex : Female	Financial Category : جوستاف روسي - نفقة الدولة
Age : 31 Y	Department : Out Patient
Referreing Doc :	Registered by : Eman Hamdi

Bone Marrow Report

Bone Marrow Aspiration Report

BMA differential count:

Normoblast : 27
Blast : 1
Promyelocyte : 15
Meylocyte : 7
Juvenile : 10
Staff : 10
Segmented : 6
Lymphocytes : 19
Monocytes : 0
Eosinophils : 0
Basophils : 0
Plasma Cells : 7

PB Comment: leukocytosis, Lymph 62% with plasmoid lymph morphology, anemia and Rouleaux, normal platelets

BMA Comment:

Normocellular marrow for age

Normal erythropoiesis in count and maturation ,Normal granulopoiesis and normal megakariopoiesis and thrombopoiesis

Normal plasma cells.

Normal lymphocytes.

Abnormal Infiltration: 7% plasma cells, many broken cells.

Conclusion:

Referred patient as H.L follow up, Normocellular marrow for age. Recommendations:

Correlation with BMB and marker study.

Signature:

Dr.Hend Abd Elhady, M.D
Ass. Prof. of Hematopathology, NCI

Verified by :

Patient Name : الأستاذ / نورمان حربي عمر على	Request No. : 13001790
Patient No. : 29401190103505	Request Date : 06/11/2025
Path No. : 3408/25	Verification Date : 02/12/2025
Sex : Female	Financial Category : جوستاف روسي - نفقة الدولة
Age : 31 Y	Department : Out Patient
Referring Doc :	Registered by : Mrs.Mayada Hegazy

Pathology Report

Clinical data

Suspected lymphoma, generalized lymphadenopathy , left axillary lymph node enlargement

Nature of specimen

Excision biopsy

Gross specimen

Fibrofatty tissue piece measured 3.5x2.5x1.5, entangling multiple (7) lymph nodes , largest 2x1.5x0.5 cm.

Technique:

- Sections were prepared from the paraffin block, processed in Bench Mark ULTRA machine (Ventana) and stained by rabbit monoclonal antibodies using ultra view HRP DAB as chromogen and hematoxylin as counter stain. The sections treated against: KI67 (30-9), PAX5 (SP34), CD15 (MMA), CD3 (2GV6), CD30 (Ber-H2), CD20 (L26).

Microscopic examination

Microscopic examination revealed normal nodal tissue with patent subcapsular sinuses . The lymphoid follicles are hyperactive and have prominent germinal centers. There is no evidence of specific granuloma or neoplasia in sections examined.

IPT :

CD20 :Positive in follicles

CD3 :Positive in interfollicular areas spacing follicles

CD30 :Totally negative

CD15 :Positive in granulocytes (eosinophils, monocytes , basophils)

PAX5 :Positive in germinal centers

KI67 :Increased in germinal centers

Verified by :

Dr.Heba Hasan

Printing Date : 09/12/2025 11:13:53

Page 1 of 2

Printing By : Mrs.Mayada Hegazy



Patient Name	: الأستاذ / نورهان حربي عمر على	Request No.	: 13001790
Patient No.	: 29401190103505	Request Date	: 06/11/2025
Path No.	: 3408/25	Verification Date	: 02/12/2025
Sex	: Female	Financial Category	: جوستاف روسي - نفقة الدولة
Age	: 31 Y	Department	: Out Patient
Referring Doc :		Registered by	: Mrs.Mayada Hegazy

Pathology Report

Diagnosis

Left axillary lymph node , excision biopsy :

Reactive follicular hyperplasia .

Signature

Prof. Dr. Gamal Abdelatty
Dr.Heba Hassan

Verified by :
Dr.Heba Hasan



Name : Nourhan Harbi Omar Ali

Date : 15-02-2026 11:03

Branch : Nasreity

Medical No. : 208795

Whole body PET/CT EXAMINATION:

Clinical History:

Female patient diagnosed with managed lymphoma (No available previous pathological data).

PET/CT requested for restaging assessment.

Technique:

For the aforementioned patient we have done a total body PET scan one hour after IV injection of 370 MBq of 18 F-FDG. MSCT was performed using GE Discovery IQ simultaneously and used for attenuation correction, anatomical localization and diagnosis. Max. variant of SUV; a Semi-quantitative analysis would be done for selected ROIs and the normal threshold is <2.5.

Report:

Limited comparison to last PET/CT report dated 27-9-2025.

Hepatic Reference SUVmax~2.6

Reference mediastinal blood pool SUVmax~1.5.

Head and Neck:

* Metabolic progression of previously reported FDG avid nasopharyngeal mucosal thickening of SUVmax~11.5 (Was 5).



* Mild metabolic progression of the previous multiple FDG avid bilateral submandibular, upper/lower deep cervical with current bilateral supraclavicular, posterior cervical and Lt infraclavicular LNs, largest still measures~1.8 x 1.5 cm of SUVmax~6.1 (Was 3.8).



* Normally diffuse brain activity with no hypermetabolic brain lesions.

* Physiologic FDG uptake is seen in the oropharynx, salivary glands and larynx, showing normal appearance.

* Physiologic FDG uptake is seen in submandibular and parotid salivary glands, showing normal appearance.

* Otherwise, Normal size, enhancement & symmetrical appearance of thyroid glands with no FDG avid lesions detected.

* Normal appearance of the neck muscles & vessels.

Name : Nourhan Harbi Omar Ali

Date : 15-02-2026 11:03

Branch : Nasr city

Medical No. : 208795

Chest:

- * Metabolic regression of the previous FDG avid small bilateral axillary LNs, still reaching~1.5 cm of SUVmax~3.7 (Was 8).



- * Mild morphological progression of the previous multiple FDG avid mediastinal retrocaval, pre-vascular, subcarinal and anterior mediastinal LNs, largest measures~2.4cm with SUVmax~4.3 (Was 1.4 cm of SUVmax~3.8).



No FDG avid lung nodules or masses detected.

- * No metabolically active pericardial or pleural lesions detected.
- * Physiologic FDG uptake is noted in the heart, showing normal shape.

Abdomen and Pelvis:

- * Rather stationary course of the previous multiple FDG avid abdomino-pelvic celiac, splenic hilar, porta-hepatis, porto-caval, para-aortic, aortocaval, almost all iliac groups and bilateral inguino-femoral LNs, the largest measures~2.5x1.2cm with SUVmax~6.9 (Was 3 x 1.4 cm of SUVmax~5.9).



- * Physiologic diffuse FDG uptake is seen over average sized liver with homogenous parenchymatous texture. No FDG avid focal lesions detected. Patent homogeneously enhancing portal vein and its main branches as well as its main tributaries. No intra-hepatic or extra-hepatic biliary radicle dilatation.
- * Still seen enlarged spleen with diffuse heterogenous FDG involvement of SUVmax~4.9 (Was 5.8)? Infiltration.
- * Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes. Physiologic FDG excretion is seen in the kidneys and bladder.

Name : Nourhan Harbi Omar Ali	Date : 15-02-2026 11:03
Branch : Nasrelty	Medical No. : 208795

- * Normal appearance of the pancreas, aorta, I.V.C and both supra-renals with no FDG avid lesions detected.
- * No FDG avid ascites or abdominal collections noted.



Musculoskeletal:

- * Normal FDG activity is seen over the axial skeleton. No focal active blastic or lytic bone lesions detected.
- * Still noted diffuse increased marrow activity of the examined axial and proximal appendicular skeleton, for clinico-laboratory correlation.

Conclusion:

Follow up PET-CT study shows:

- Metabolic progression of previous FDG avid nasopharyngeal mucosal thickening.
- Mild metabolic progression of previous multiple FDG avid cervical as well as mediastinal nodal lesions.
- Stationary course of the previous FDG avid infra-diaphragmatic nodal lesions as well as suggested splenic infiltration.

Mariam Ali, MSc

Thank you for referral

Walid Omar, MD.

Prof. of Nuclear Medicine

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about 1000
HARBY, NORHAN Patient ID: 124002629 DOB: 19-Jan-1994 Date: 07-Feb-2024
Patient Lastname: Harby
Patient Firstname: NORHAN
HIS Patient ID: 124002629
RIS Patient ID: 348467
Date of Birth: 19/01/1994

Procedure: 93015, FDG PETCT
Medical Question:
Clinical Comment:

===== REPORT TEXT =====

Patient Name: Norhan Harby Omar
ID: 124002629
Whole Body PET/CT (7/2/2024)

History and clinical findings:
" 29 years old female patient has history of HL for initial assessment.

TECHNIQUE:

" Approximately 1 hour following intravenous injection of the radiopharmaceutical fluorodeoxyglucose (FDG), PET-CT was acquired from skull to mid-thigh with the arms extended above the head. CT was acquired without contrast.

Report

Head and Neck:

- * Metabolically active FDG avid multiple bilateral upper and lower deep cervical, bilateral submandibular and bilateral supra and infra clavicular LNs; the largest measuring 2x1.4 cm with SUVmax~6.7
- * Metabolically active (FDG avid) nasopharyngeal thickening is noted with SUVmax~6.8 of equivocal nature for clinical correlation and follow up.
- * Physiologic FDG uptake is seen in the oropharynx, salivary glands, and larynx.
- * The brain shows uniform tracer distribution with NO hypermetabolic focal lesions.

Chest:

- * Metabolically active FDG avid multiple bilateral sub-pectoral, inter-pectoral and axillary LNs; the largest measuring 2.7x1.4 cm with SUVmax~6.1.
- * Metabolically active FDG avid bilateral sub-cm brachial nodules with SUVmax~3.3.
- * Metabolically active FDG avid superior mediastinal, pre-tracheal, retro-caval, pre-vascular, bilateral internal mammary, carinal, sub-carinal and epi-phrenic LNs the largest measuring 2.3x1.7 cm with SUVmax~6.8.
- * No FDG avid lung nodules
- * There are no active pleural or pericardial abnormalities.
- * Physiologic FDG uptake is noted in the heart.

Abdomen and Pelvis:

- * Metabolically active FDG avid abdominal and pelvic LNs are seen at peri-aortic, gastro-hepatic, per-pancreatic, porta-hepatis, aorto-caval, porto-caval, mesenteric, bilateral common, internal and external iliac, bilateral inguino-femoral LNs; the largest measuring 3.1 cm with SUVmax~8.5
- * Splenomegaly with diffuse inhomogeneous FDG uptake with SUVmax~ 5.6 likely infiltrative.
- * Few FDG avid small peritoneal nodules are noted beneath anterior abdominal wall at left lumbar